

HIV PRE-EXPOSURE PROPHYLAXIS (PrEP): ROUTINE CARE

Assess the patient starting HIV PrEP at baseline, within 28 days and then 3 monthly.

Assess	When to assess	Note
Symptoms	Every visit	Manage symptoms as on symptom pages. If cough or fever $\geq$ 2 weeks, unexplained weight loss or night sweats, exclude TB ↗ 92.
STI symptoms	Every visit	Screen for STI: if discharge, rash, itch, lump/s, ulcer/s ↗ 49.
Adherence	Every visit	Ask about pill taking pattern and missed doses. If difficulty with adherence ↗ 173. If needed, align visits with baby's routine care or EPI visit.
Side effects	Every visit	Ask about side effects of medication (see below). Reassure these usually resolve within a few weeks.
Alcohol/drug use	Every visit	In the past year, has patient: 1) drunk $\geq$ 4 drinks/session, 2) used illegal drugs or 3) misused prescription or over-the-counter medications? If yes to any ↗ 142.
Sexual health	Every visit	If risky sexual behaviour: new or multiple partner/s, uses condoms unreliably, has sex under influence of alcohol/drugs, give safe sex advice.
Family planning	Every visit	- Exclude pregnancy ↗ 157. Assess patient's contraceptive needs ↗ 154. There are no interactions between PrEP and hormonal contraception. - Align contraception visit with PrEP visit, if possible.
HIV test	Baseline ² , at 1 month, then every 3 months	If positive, stop HIV PrEP and give routine HIV care ↗ 111. If negative, continue HIV PrEP.
HBsAg	Baseline	- If positive, continue PrEP, take blood for ALT and refer doctor to monitor liver function: if ALT > 2 times upper limit of normal, refer. - If negative, continue PrEP, consider giving hepatitis B vaccine ↗ 120.
Creatinine (eGFR)	- If $\geq$ 30 years: baseline - If diabetes/hypertension: baseline, then yearly - If pregnant: baseline, at 3 and 6 months.	- If not pregnant: if eGFR < 30, refer same day. If eGFR < 50, repeat creatinine (eGFR) on a separate day: if repeat eGFR is $\geq$ 50, continue PrEP. - If repeat eGFR still < 50, stop PrEP and discuss with doctor/HIV hotline ↗ 178. - If pregnant: if creatinine > 85, repeat creatinine on a separate day: if repeat creatinine is $\leq$ 85, continue PrEP. - If repeat creatinine still > 85, stop PrEP and discuss with doctor/HIV hotline ↗ 178.
Syphilis	At baseline	Rapid tests are preferable, as results are immediately available. If positive ↗ 53.

Advise the patient on/starting HIV PrEP

- If starting, advise that daily HIV PrEP is only effective after taking it for at least 7 days. Emphasize importance of condom use until PrEP effective.
- Advise that HIV PrEP can be started and stopped according to risk. Discuss with nurse/doctor before stopping. Advise patient that HIV PrEP is not treatment for HIV and to avoid sharing medication.
- Emphasize that HIV PrEP does not prevent pregnancy or other STIs. Advise to avoid unprotected sex. Encourage reliable use of condoms and supply male and female condoms and lubricants. Offer referral for male circumcision. Explain the need for regular HIV testing and advise partner/s to test for HIV. Help patient to plan and set goals for behavior changes that may reduce his/her risk.
- Support adherence: advise that s/he needs to take the PrEP medication every day for it to be effective in preventing HIV infection.
 - Suggest patient uses a weekly pillbox and/or reminders like cell phone alarms or calendar checklist
 - If missed pill, advise to take it as soon as s/he remembers within 24 hours and not take more than 1 pill in one day.
 - Encourage patient to identify friends/family who can support adherence. Refer patient to local/online support group or adherence club, if available.

Treat the patient on/starting HIV PrEP

- If starting, give medication for 1 month. If already on HIV PrEP, give medication for 3 months. Complete PrEP Clinical Form at each visit.

Medication	Dose	Side effects
Tenofovir/emtricitabine (TDF/FTC)	300/200mg once daily	Nausea, headache, tiredness, diarrhoea, depression, abnormal dreams, vomiting, rash, problems sleeping, changes in appetite.

Review after 1 month, sooner if side effects develop. Then 3 monthly: for HIV test and prescription of medications.

Decide when to stop HIV PrEP

If patient is no longer at high risk of HIV and wishes to stop PrEP, plan with patient to stop PrEP at least 7 days after last potential HIV exposure. If HBsAg-positive, avoid stopping PrEP and refer instead.

¹One drink is 1 tot of spirits, or 1 small glass (125mL) of wine or 1 can/bottle (330mL) of beer. ²If starting PrEP is delayed, repeat HIV test on same day that PrEP is started ↗ 110.